CAPHE

California Association of Public Health Economists

Founding Member Survey

Mission, Audience, and Priorities

Results as recorded through August 26, 2026.

Fielded August 24 to 26, 2026

6 responses recorded · 4 contained answers

All counts use the 4 responses with answers.

RESPONSE BASE

What the Export Contains

6	4	2	6
responses recorded August 24 to 26, 2026	contained answers used for all counts	no answers recorded every question blank	marked finished progress at 100%

On the two blank responses

Both were recorded as finished with progress at 100%, and contain no answer to any question.

Their recorded durations were 567 seconds and 93 seconds.

Source: Qualtrics export, CAPHE Founding Member Survey, August 26, 2026, 10:57.

Most Important Problem to Address

Choose up to two. n = 4 responses containing answers.

Health economists working in California public health lack a peer network, shared learning, and professional community.	4
Health-economic work is not sufficiently integrated into public health planning, accreditation, policy, and funding decisions.	2
Public health leaders do not consistently see health economics as relevant to their decisions.	1
Health economics is too often understood narrowly as causal inference or formal cost-effectiveness analysis.	1

Options that received no selections are listed at the end of the deck.

SECTION 2 · PRIMARY MISSION

Mission for the Next One to Three Years

Choose one. n = 4 responses containing answers.

C. Increase institutional demand for health economics. Help public health leaders understand why and how health economics improves decisions, strengthens accountability, and supports the case for prevention.	2
B. Build a statewide applied health-economics community of practice. Connect state and local health economists to share methods, tools, examples, and peer support.	1
E. Advance a broader public-health economics field in California. Create shared standards, visibility, partnerships, and infrastructure for health economics across California public health.	1

Options that received no selections are listed at the end of the deck.

SECTION 2 · OPEN RESPONSE

What Is Missing From the Mission Chosen

Optional question. 3 of 4 respondents answered.

Respondent who selected mission E

"I don't know how to incorporate it, but the 'increase demand' piece from option C is important and missing from E. I think E better captures the breadth of what CAPHE should focus on, but visibility is not the same as showing value / increasing institutional demand."

Respondent who selected mission C

"If health economics is essential, and if it is absent, we should try to understand what is being used in place of the essential health economics and whether (or if!) it fails."

Quoted as submitted. Continued on the next slide.

SECTION 2 · OPEN RESPONSE

What Is Missing From the Mission Chosen

Continued.

Respondent who selected mission B

"Economics is a study of human progress and the creation of wealth (incl. health).

Economics is not a regulatory tool. Health economics is not about finding more ways for the government to be involved in more aspects of healthcare or shaping health outcomes.

Health economics trains us to recognize trade-offs, trace out unintended spillovers of health programs, deploy public health $$ in their best uses, and reduce transactions costs in healthcare markets. This is how we must apply the discipline."

Quoted as submitted. One respondent left this question blank.

SECTION 3 · PRIMARY AUDIENCE

Whose Decisions the Group Aims to Influence

Choose up to two. n = 4 responses containing answers.

Local health department senior leaders and executives	3
State public health leaders and program leadership	3
Local health department program directors and managers	1
Community-based organizations and cross-sector partners	1

Options that received no selections are listed at the end of the deck.

Who Makes Up the Group Itself

Choose up to two. n = 4 responses containing answers.

Local health department health economists, analysts, and evaluators	2
University and research partners	2
State health department economists, analysts, and policy staff	1
Emerging public health economists and trainees	1
Other (please describe)	1

Text entered under “Other”

“The first two: local and state health department health economists, analysts, and evaluators / policy staff.”

Options that received no selections are listed at the end of the deck.

SECTION 4 · STRATEGIC ROLE

Most Important Contribution, Ranked

Rank the top three, 1 = most important. n = 4 responses containing answers.

Strategic role	Ranked 1	Ranked 2	Ranked 3	In top 3
Toolkit builder	2			**2**
Community of practice	1	1		**2**
Technical-assistance provider			2	**2**
Translator	1			**1**
Field advocate		1		**1**
Demonstration partner		1		**1**
Training catalyst			1	**1**
Connector				**0**

Cells show how many respondents assigned that rank. Three respondents ranked three roles; one ranked only a first choice.

Activities the Group Could Credibly Sustain

Choose up to three. n = 4 responses containing answers.

Community of practice	3
Translator	3
Field advocate	1
Technical-assistance provider	1

Selections per respondent

Two respondents selected three activities each; the other two selected one activity each.

Options that received no selections are listed at the end of the deck.

The Group's Role in Relation to Them

Choose up to two. n = 4 responses containing answers.

Develop case studies demonstrating the recommendations in practice.	2
Advise on content, practical relevance, and California examples.	2
Champion the recommendations with CDPH and other partners.	1
Curate or disseminate tools and resources developed by CDPH or partners.	1
Seek funding or partnerships to pilot a limited consultation model.	1

Options that received no selections are listed at the end of the deck.

Balance Between Agencies and Members

Choose one. n = 4 responses containing answers.

We should first establish a clear professional community, then broaden to external-facing work.	2
Our priority should be helping leaders and agencies use economics, even if the group mainly convenes economists behind the scenes.	1
We should balance the two equally.	1

Options that received no selections are listed at the end of the deck.

SECTION 6 · SCOPE

How Broadly to Define Health Economics

Choose one. 3 of 4 respondents answered.

Broadly: any systematic use of economics to understand value, incentives, resource allocation, equity, financing, and public-health decision-making.	2
Moderately: the above plus economic burden, financing, behavioral economics, and policy analysis.	1

One respondent left this question blank. Options that received no selections are listed at the end of the deck.

SECTION 6 · OPEN RESPONSE

Greatest Risk if Too Externally Focused

Open response. 2 of 4 respondents answered.

"Lack of growth, leading to limited ability to influence adoption by state / local agencies due to not having the right contacts or visible presence."

"We will become wildly popular and make lose our focus on academic economics."

Quoted as submitted. Two respondents left this question blank.

SECTION 6 · OPEN RESPONSE

Greatest Risk if Primarily a Member Network

Open response. 2 of 4 respondents answered.

"We live in a bubble, agreeing that health economics can be an important contributor to policy and budgeting decisions, but with no strategy or implementation efforts to make it happen."

"We will have great discussions but not improve the lives of our members (outside of the great discussions)."

Quoted as submitted. Two respondents left this question blank.

SECTION 6 · OPEN RESPONSE

The Two Risks, Side by Side

6.3 and 6.4 compared. The same 2 of 4 respondents answered both questions.

6.3 If the group becomes too externally focused

Respondent A

"Lack of growth, leading to limited ability to influence adoption by state / local agencies due to not having the right contacts or visible presence."

Respondent B

"We will become wildly popular and make lose our focus on academic economics."

6.4 If the group remains primarily a member network

Respondent A

"We live in a bubble, agreeing that health economics can be an important contributor to policy and budgeting decisions, but with no strategy or implementation efforts to make it happen."

Respondent B

"We will have great discussions but not improve the lives of our members (outside of the great discussions)."

Quoted as submitted. Two respondents left both questions blank.

Ratings of the Draft Statement

Rated 1 = low to 5 = high. n = 4 responses containing answers.

3.5	3.8	3.3
Clarity	Importance	Feasibility
scores: 4, 5, 3, 2	scores: 4, 5, 4, 2	scores: 3, 5, 2, 3

Long version: "…a statewide community of practice that helps public health agencies make more informed, equitable, and transparent decisions by connecting people to applied health-economics expertise, translating economic methods into usable tools and examples, and demonstrating the value of health economics in public health."

Short version: "We help California public health agencies use health economics to make better, more equitable decisions about limited resources."

Means are the arithmetic average of the four scores.

SECTION 7 · OPEN RESPONSE

What Would Improve the Statement

Optional question. All 4 respondents answered.

"Not sure why the longer version says 'connecting people' to applied HE expertise. Shouldn't 'people' be 'them' instead? ('Them' referring to public health agencies.)"

"I would drop equity and equitable because those words are never well-defined. In my experience, it never makes the analysis more transparent or inquiring."

"Helping California's public health leaders use health economics to make better decisions."

"Better, more equitable than what?"

Quoted as submitted.

SECTION 8 · WHAT SUCCESS LOOKS LIKE

Outcomes That Would Show Success at 18 Months

Choose exactly two. n = 4 responses containing answers.

Members have a reliable peer network for consultation and collaboration.	3
Public health leaders can clearly describe how health economics can support their decisions.	2
Agencies have access to practical California-relevant tools, examples, and data sources.	1
The group has produced two to three visible examples showing how health economics changed a real decision.	1
CDPH or another partner has adopted or funded a structured technical-assistance or consultation model.	1

Options that received no selections are listed at the end of the deck.

OPTIONS RECEIVING NO SELECTIONS

Questions 1.1 to 2.1

Page 1 of 4. n = 4 responses containing answers.

1.1 Most important problem

Local health agencies do not have enough access to health-economics expertise.

Agencies lack practical, reusable tools for budgeting, prioritization, ROI, cost analysis, and decision-making.

Local and state agencies need better ways to communicate the value of prevention and public-health investment.

Other

2.1 Primary mission

A. Make health economics usable in public health decisions.

D. Build workforce capability.

F. Another mission

OPTIONS RECEIVING NO SELECTIONS

Questions 3.1 to 4.1

Page 2 of 4. n = 4 responses containing answers.

3.1 Primary audience

Analysts, evaluators, epidemiologists, planners, and policy staff

Finance, budget, grants, and contracts staff

PHAB accreditation and quality-improvement staff

Public health economists and people doing economics-adjacent work

Policymakers, funders, and public health advocates

Other

3.2 Core participating community

Consultants and technical-assistance providers

Local health department program, planning, finance, and accreditation staff

Other, beyond the free-text entry already shown

4.1 Not ranked by any respondent

Connector

OPTIONS RECEIVING NO SELECTIONS

Questions 4.2 to 6.1

Page 3 of 4. n = 4 responses containing answers.

4.2 Activities the group could sustain

Connector

Toolkit builder

Training catalyst

Demonstration partner

5.1 Role on the assessment recommendations

Help recruit and organize members to provide office hours or peer consultation.

Focus elsewhere; the group should not lead these activities.

Other

6.1 Balance between agencies and members

Our priority should be supporting and strengthening the health economists themselves; agency impact follows from that.

OPTIONS RECEIVING NO SELECTIONS

Questions 6.2 to 8.1

Page 4 of 4. n = 4 responses containing answers.

6.2 Scope of health economics

Narrowly: economic evaluation, cost-effectiveness, ROI, and resource allocation.

Not sure.

8.1 Outcomes at 18 months

Health economics is more routinely incorporated into PHAB, CHNA/CHIP, strategic planning, budgeting, or program reviews.

The group has grown into a sustainable statewide community with shared leadership and a clear identity.

The group has influenced policy, funding, or organizational expectations regarding health economics.

Source: Qualtrics export, CAPHE Founding Member Survey, August 26, 2026, 10:57.